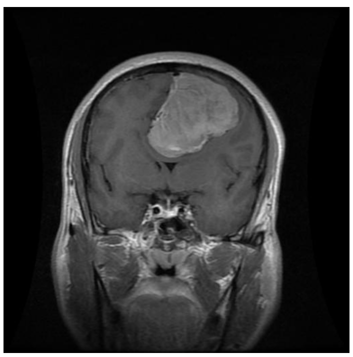
MEDVISION AI CLINICAL ASSISTANT

REPORT ID: MV-000004
DATE: 2026-07-16 00:34

Patient Name:	joey	**Scanner Resolution:**	224 x 224 px (Resized)
Patient Email:	joey@gmail.com	**Inference Engine:**	EfficientNetB0 (ImageNet)
Clinical System:	MedVision AI v1.0.0	**Processing Speed:**	69035.12 ms

SCAN ANALYSIS & COMPUTER-AIDED DETECTION (CAD)



CLASSIFICATION RESULT: **MENINGIOMA**

MODEL CONFIDENCE: **100.0%**

PROBABILITIES BREAKDOWN:

Glioma: 0.0% ■■■■■■■■■■
Meningioma: 100.0% ■■■■■■■■■■
No Tumor: 0.0% ■■■■■■■■■■
Pituitary: 0.0% ■■■■■■■■■■

CLINICAL INTERPRETATION & EXPLANATION (EDUCATIONAL)

A Meningioma is a primary central nervous system tumor that arises from the meninges—the three layers of protective membranes covering the brain and spinal cord. It is the most frequently diagnosed primary brain tumor in adults, constituting about 37% of all primary brain tumors. The vast majority of meningiomas (around 80-90%) are classified as benign (WHO Grade I) and grow slowly. However, because of their slow development, they can reach significant size before presenting symptoms such as headaches, seizures, or focal neurological deficits due to mass effect. Treatment ranges from active monitoring ("watchful waiting") for small, asymptomatic tumors, to neurosurgical resection and radiation therapy for larger or progressive lesions.

CRITICAL CLINICAL DISCLAIMER:

This report is automatically compiled by an artificial intelligence image processing system using Deep Learning (EfficientNetB0 Architecture). The diagnostic screening is meant strictly for research, educational support, and preliminary triage. It does NOT constitute formal medical advice, diagnostic confirmation, or treatment recommendation. All findings MUST be correlated with patient history, physical examination, and reviewed formally by a qualified neuro-radiologist or neurosurgeon before initiating clinical decisions.

AI System Signature:
MedVision AI Core Engine V1

Clinical Verification Signature:

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Dr. / MD Neuroradiologist